

BLÖDSINN!

INITIATING INITIATING
EPISTEMIC SABOTAGE
PROTOCOL...

Georg von Westphalen

RESIST THE MEDICAL
INDUSTRIAL
COMPLEX!

MEDKAYFAB: GASTROENTEROLOGY WRESTLING

NO GODS,
NO MASTERS,
NO PEER REVIEW!

FIGHT THE POWER
WITH FAKE NEWS
REAL SCIENCE!

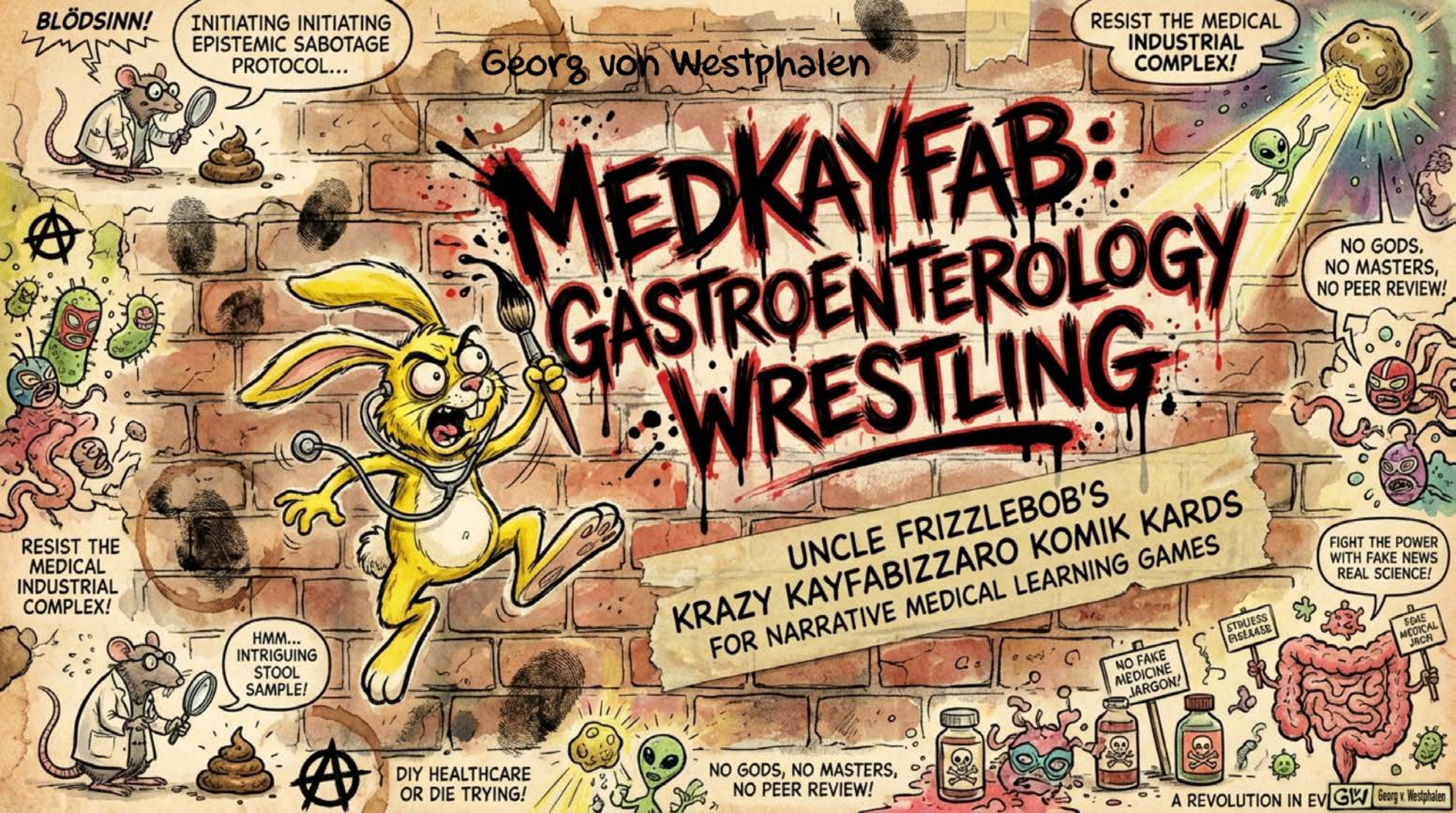
UNCLE FRIZZLEBOB'S
KRAZY KAYFABIZZARO KOMIK KARDS
FOR NARRATIVE MEDICAL LEARNING GAMES

HMM...
INTRIGUING
STOOL
SAMPLE!

DIY HEALTHCARE
OR DIE TRYING!

NO GODS, NO MASTERS,
NO PEER REVIEW!

A REVOLUTION IN EV **GW** Georg v. Westphalen



GORD Gordon | Gastro-Oesophageal Reflux Disease

STAT: AET
>8%



POWER: TLOS-R-Breach — Bypasses all crural diaphragm defenses via GABA-B mediated vago-vagal reflexes.

LORE: PPIs must be taken 30-60 minutes before meals to target the $H^+/K^+-ATPase$ in activated parietal cells.

Barrett's Barry | Metaplasia & Dysplasia

STAT: 0.3%/yr
Malignancy



POWER: Columnar Shift — Adapts to acid, initiating the $p16 \rightarrow p53 \rightarrow SMAD4$ carcinogenic sequence.

LORE: Confirmed by columnar-lined oesophagus ≥ 1 cm with intestinal metaplasia; requires surveillance or RFA.

Achalasia Alice | Failed LOS Relaxation

STAT: IRP
>15 mmHg

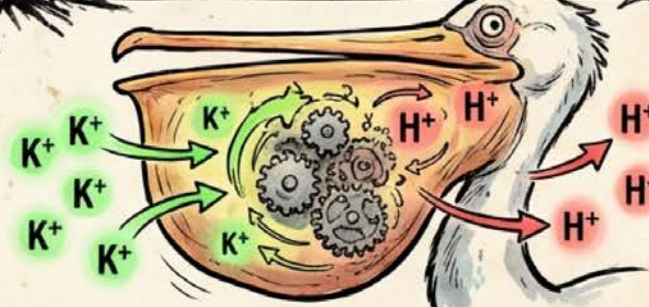


POWER: NO-Depletion — Prevents all enemy relaxation effects, trapping food and liquid in permanent spasm.

LORE: Dysphagia to solids AND liquids from the onset, unlike carcinoma which is solids-first.

Gastric Gus | Parietal Cell Biology

STAT: pH 0.8
Gradient



POWER: Alkaline Tide — Exchanging HCO_3^- for Cl^- at the basolateral membrane after massive acid secretion.

LORE: Intrinsic factor, secreted exclusively by parietal cells, is the absolute only source of B12 absorption.

PUD Paulo | Peptic Ulcer Disease

STAT: pH 1→7
Gradient



POWER: **COX-1 Blockade** — Removes cytoprotective PGE2, destroying mucosal blood flow and restitution.

LORE: Enteric coating fails to protect against ulcers because NSAID damage is systemic, not just topical.

Zollinger-Ellison Zack | Gastrinoma

STAT: Fasting
pH <2



POWER: **Secretin Paradox** — IV Secretin causes a paradoxical gastrin rise ≥ 120 pg/mL, bypassing negative feedback.

LORE: Massive acid hypersecretion inactivates pancreatic lipase, causing steatorrhea alongside multiple ulcers.

Gastroparesis Greta | Gastric Motility

STAT: >10%
Retention @ 4h



POWER: **Vagal Neuropathy** — Halts the migrating motor complex, preventing the Phase III housekeeper wave.

LORE: Hyperglycaemia directly inhibits antral motility; glycaemic control is the critical first step.

Gastric Cancer Gina | Gastric Adenocarcinoma

STAT: 80% non-
cardia (*H. pylori*)

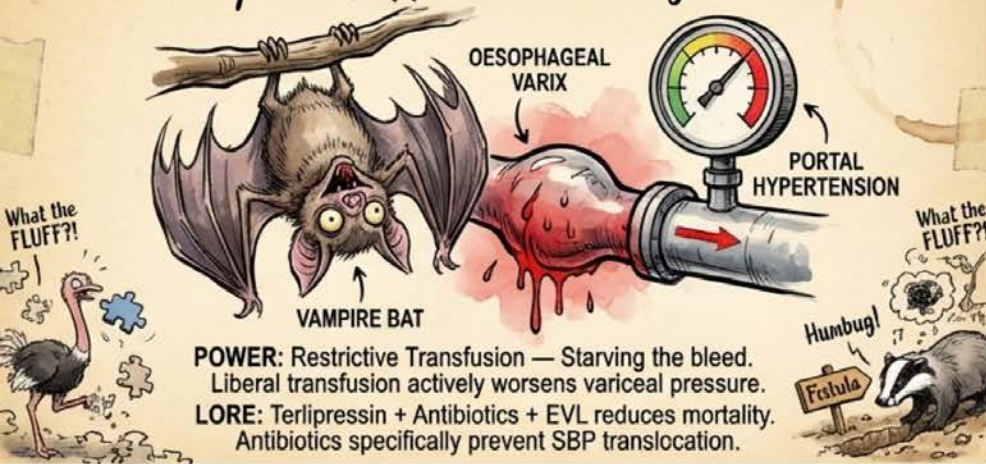


POWER: **CDH1 E-Cadherin Loss** — Triggers hereditary diffuse gastric cancer and linitis plastica.

LORE: TCGA molecular profiling dictates treatment; HER2 amplification uniquely allows for trastuzumab targeting.

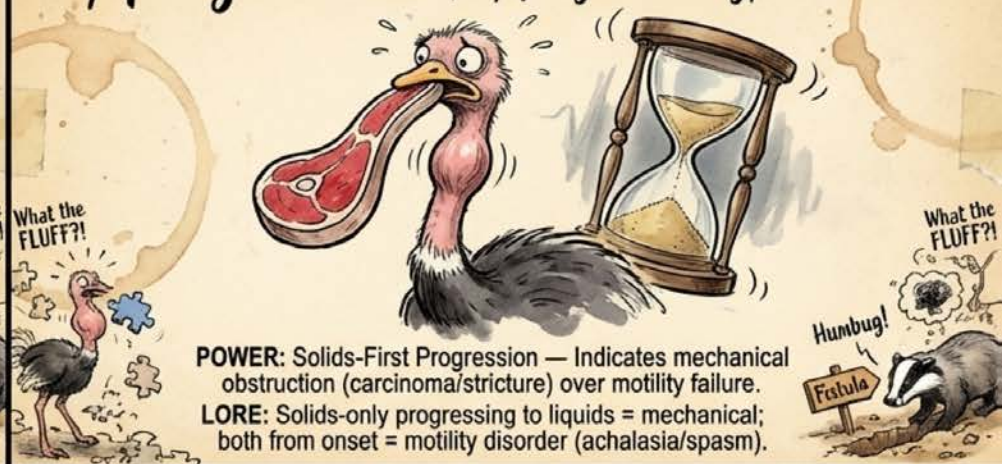
UGIB Ulysses | Upper GI Haemorrhage

STAT: Target Hb 70-80 g/L



Dysphagia Dmitri | Dysphagia Aetiology

STAT: 95% Squamous (Proximal)



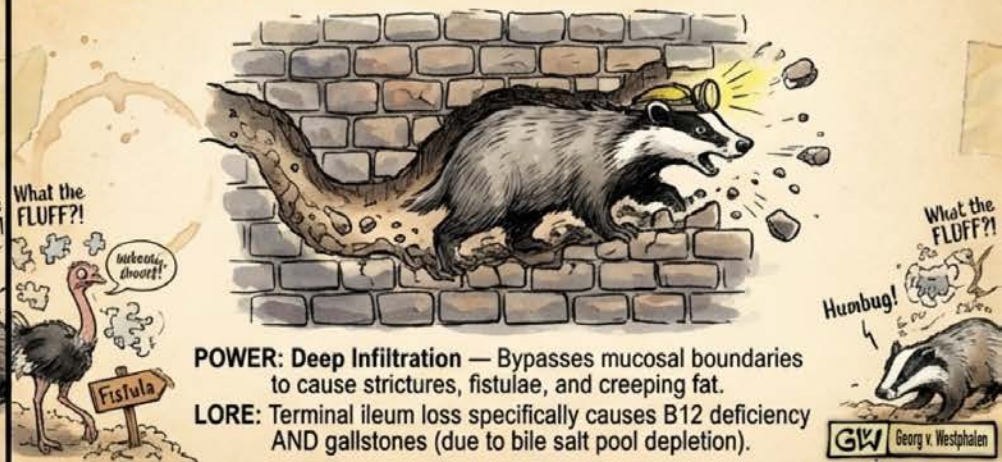
Coeliac Celia | Gluten Immunopathology

STAT: HLA-DQ2 in 95%



Crohn's Charlie | Transmural Inflammation

STAT: 40% Ileocaecal



Ulcerative Colitis Uma | Mucosal Inflammation

STAT: Truelove-Witts ASC



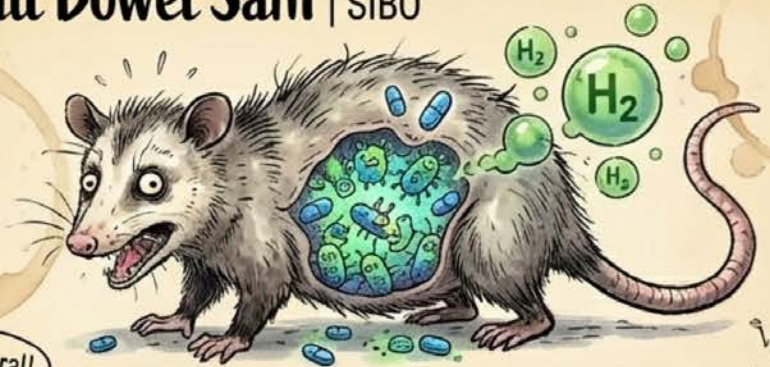
POWER: Crypt Abscess — Neutrophilic assault restricted entirely to the superficial mucosal layer. No skip lesions.

LORE: Patients with both UC and PSC require annual colonoscopy surveillance from diagnosis.

Mucosal!

Small Bowel Sam | SIBO

STAT: $>10^5$ CFU/mL



POWER: Bile Deconjugation — Ruins micelle formation, causing massive fat malabsorption and steatorrhea.

LORE: Bacteria consume B12 but produce folate; SIBO characteristically presents as B12 deficiency with elevated folate.

Transmural!

Carcinoid Carlos | Carcinoid Syndrome

STAT: 5-HIAA >25 $\mu\text{mol/d}$



POWER: Hepatic Bypass — Syndrome only occurs when liver metastases evade MAO-A detoxification.

LORE: The left heart is spared from fibrotic valve plaques because the lungs contain MAO-A which destroys serotonin.

SYSTEMIC CIRCULATION

Short Bowel Sebastian | Short Bowel Syndrome

STAT: $<200\text{cm}$ Remaining



POWER: GLP-2 Hyperplasia — Triggers massive enterocyte adaptation and bowel dilatation to salvage nutrients.

LORE: Retaining the colon allows colonic bacteria to ferment carbohydrates into SCFAs, salvaging up to 1500 kcal/day.

GW

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Bile Acid Bertha | Enterohepatic Circulation

STAT: 95%
Reabsorbed (Ileum)



POWER: FXR Negative Feedback — Suppresses CYP7A1 via FGF-19 to stop cholesterol conversion.
LORE: Type 2 BAM occurs when FGF 19 fails, causing unsuppressed bile acid synthesis and severe watery diarrhea.

Gut Motility Marta | Enteric Nervous System

STAT: 500
Million Neurons



POWER: Serotonin Peristalsis — EC cells release 95% of the body's serotonin to drive motility.
LORE: Ondansetron works as an antiemetic specifically by blocking 5-HT3 receptors on vagal afferents.

LGIB Larry | Lower GI Haemorrhage

STAT: 80%
Spontaneous Stop



POWER: Heyde's Cleavage — Aortic stenosis high shear stress destroys HNW vWF, causing angiodysplasia Meeds.
LORE: Diverticular bleeding is arterial, painless, and large-volume; mostly occurring in the right colon.

Colorectal Cancer Colin | Adenoma-Carcinoma

STAT: 85%
CIN Pathway

APC → KRAS → SMAD4 → TP53



POWER: KRAS Resistance — Constitutive pathway activation entirely ignores upstream blockade.
LORE: Cancers with a KRAS mutation are intrinsically resistant to anti-EGFR therapies like cetuximab.

Diverticular Dina | Diverticular Disease

STAT: 60% Prevalence at 80y



POWER: Hinchey Rupture — Progresses from micro-perforation to purulent/faecal peritonitis requiring Hartmann's.

LORE: Recent RCTs confirm antibiotics are not necessary for uncomplicated acute diverticulitis.

Microscopic Colitis Mia | Invisible Inflammation

STAT: >10µm Collagen Band



POWER: NSAID Trigger — Chemical induction of chronic, watery, non-bloody nocturnal diarrhoea in older women.

LORE: A perfectly normal colonoscopy in chronic watery diarrhoea mandates random biopsies to check for this.

Colonic Polyps Perry | Neoplastic Lesions

STAT: >20mm = 30% Malignant



POWER: Sessile Serration — Bypasses the traditional APC pathway via BRAF and MLH1 methylation (MSI).

LORE: Sessile serrated lesions are particularly dangerous due to flat morphology and rapid malignant potential.

IBD Extra-Intestinal Eva | Systemic Manifestations

STAT: HLA-B27 Associated

Multi-Organ Chimera

POWER: Pathergy — Trauma accelerates destruction; surgical debridement of pyoderma gangrenosum is disastrous.

LORE: Some manifestations parallel gut activity (erythema nodosum), while others (uveitis, AS, PSC) run completely independently.



Liver Function Louisa | The Metabolic Factory

STAT: Albumin
Half-life 21d



POWER: Phase II Glucuronidation — Renders toxic hydrophobic compounds water-soluble via UGT1A1 for biliary excretion.
LORE: INR is a much better marker of acute synthetic failure than albumin due to clotting factors' short half-lives.

Jaundice Jerome | Hyperbilirubinaemia

STAT: ALP:ALT > 3:1
(Biliary)



POWER: Urobilinogen Starvation — Biliary obstruction prevents bilirubin from reaching the gut to become stercobilin.
LORE: Dark urine + pale stools + absent urinary urobilinogen uniquely identifies complete post-hepatic obstruction.

Cirrhosis Cyrus | End-Stage Liver Disease

STAT: HVPg
≥10 mmHg



POWER: TGF- β Fibrogenesis — Permanently replaces healthy parenchyma with regenerative nodules.
LORE: First decompensation event drops median survival from 12 years to just 2 years.

HE Hector | Hepatic Encephalopathy

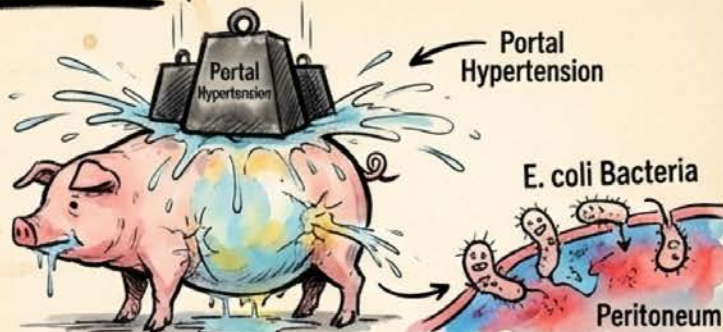
STAT: Grade I
to IV Coma



POWER: GABAergic Tone Surge — Neurosteroids and ammonia induce extreme sedation; benzos are lethal.
LORE: Never restrict protein in HE; sarcopenia worsens outcomes. Treat with lactulose and rifaximin instead.

Ascites Antonia | SBP & Ascites

STAT: SAAG ≥ 11 g/L



POWER: Opsonin Failure — Low ascites protein (<15 g/L) allows translocated E. coli to trigger Spontaneous Bacterial Peritonitis.
LORE: IV Albumin given with antibiotics for SBP drastically reduces mortality by preventing Type 1 Hepatorenal Syndrome.

NAFLD/NASH Nathan | Metabolic Liver Disease

STAT: FIB-4 > 2.67
Rule-In



POWER: PNPLA3 I148M Mutation — Halts triglyceride hydrolysis, causing massive lipid droplet accumulation.
LORE: The fastest growing cause for transplant; weight loss >10% can actually regress fibrosis.

ALD Albert | Alcoholic Liver Disease

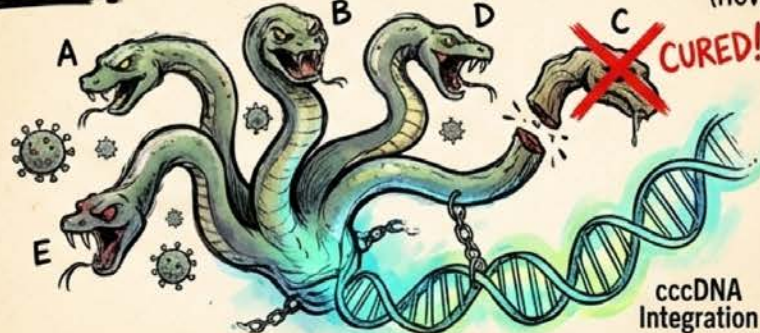
STAT: AST:ALT > 2:1



POWER: Mitochondrial Toxicity — Preferential destruction releases mitochondrial AST, causing the classic ratio.
LORE: Chronic alcohol induces CYP2E1, drastically lowering the toxic threshold for paracetamol overdose.

Viral Hep Victor | Hepatitis Viruses

STAT: >95% DAA SVR (HCV)



POWER: cccDNA Integration — HBV integrates permanently into the host genome, making true cure nearly impossible.
LORE: HCV is an RNA virus treated with DAAs for a complete cure; HBV requires lifelong suppression.

HCC
Risk

AIH/PBC Audrey | Autoimmune Liver Disease

STAT: AMA M2 95%
(PBC)



POWER: Pyruvate Dehydrogenase Cross-React — Autoantibodies destroy bile ducts while sparing hepatocytes.
LORE: Pruritus is severe due to TGR5 activation by bile salts; UDCA slows ductular damage.

DILI Davina | Drug-Induced Liver Injury

STAT: R-Ratio
(ALT/ALP)



POWER: NAPQI Necrosis — Overwhelms glutathione stores to cause massive centrilobular hepatocyte death.
LORE: NAC is only effective for paracetamol overdose if given early, before irreversible covalent binding occurs.

Haemochromatosis Harold | Iron Overload

STAT:
TSAT >45%



POWER: Hepcidin Failure — HFE mutation ruins transferrin sensing, causing unchecked intestinal iron absorption.
LORE: Causes Bronze Diabetes due to iron deposition in the pancreas; HCC risk persists even after venesection.

Wilson's Wilma | Copper Overload

STAT:
Ceruloplasmin <200



POWER: ATP7B Defect — Blocks biliary copper excretion, forcing accumulation in the basal ganglia and liver.
LORE: A disproportionately low ALP with markedly elevated bilirubin is highly characteristic of acute Wilson's ALF.

Diagnosis?!

KayfaBINGO!

GW

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Gallstones

Gertie | Biliary Syndromes

STAT: 80% Cholesterol



POWER: Supersaturation — Depleted bile acid pools or excess cholesterol cause physical crystal nucleation.

LORE: Charcot's triad becomes Reynold's pentad (adding hypotension + confusion) marking lethal septic shock.

Cholangiocarcinoma

Clare | Biliary Cancer

STAT: 50% Perihilar (Klatskin)



POWER: FGFR2 Fusion — Molecular target allowing specific inhibition with pemigatinib in intrahepatic subtypes.

LORE: PSC carries a 1500x relative risk for CCA due to chronic fibro-inflammatory duct destruction.

Pancreatitis

Pablo | Acute Pancreatitis

STAT: CRP >150 at 48h



POWER: Zymogen Activation — Premature intracellular cleavage triggers massive local and systemic inflammatory cascades.

LORE: Enteral feeding (NG/NJ) is superior to TPN as it preserves gut barrier and stops necrotic infection.

Chronic Pancreatitis

Christina | Fibrosis & Failure

STAT: Faecal Elastase <100



POWER: Type 3c Brittle Diabetes — Complete destruction of both insulin AND glucagon cells causes wild glucose swings.

LORE: B12 malabsorption occurs because the acidic duodenum (from lack of bicarb) disrupts the IF-B12 complex.

Pancreatic Cancer Petra | PDAC

STAT: <12% 5-year OS



POWER: Immune Exclusion — KRAS-driven microenvironment physically prevents drug delivery and checkpoint inhibitors.
LORE: Painless obstructive jaundice with a palpable non-tender gallbladder (Courvoisier's) is PDAC until proven otherwise.

GI Bleed Emergency Gemma | Haemorrhage

STAT: 1:1:1 MTP Ratio



POWER: Restrictive Trigger — Hb 70-80 g/L is the optimal target; pushing more fluid pops the clots.
LORE: Unlike in trauma, Tranexamic Acid (TXA) has no mortality benefit in GI bleeding (HALT-IT trial).

Gut Microbiome Mike | Dysbiosis

STAT: 38 Trillion Bugs



POWER: Colonisation Resistance — Healthy flora outcompetes *C. difficile* for space and nutrients.
LORE: Faecal microbiota transplant (FMT) boasts an 85-90% success rate by restoring the destroyed ecological niche.

Pancreatitis Complications | Walled-Off Necrosis

STAT: >4 Weeks Encapsulation



POWER: Step-Up Debridement — Endoscopic transgastric drainage dominates over primary open surgical necrosectomy.
LORE: Splenic vein thrombosis from body/tail pancreatitis causes isolated gastric varices without oesophageal varices.

Functional GI Fiona | Visceral Hypersensitivity

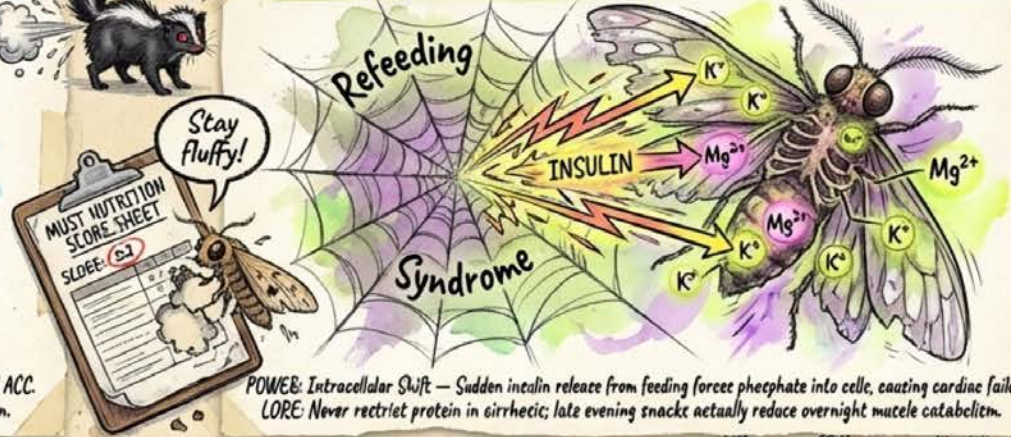
STAT: Rome IV Criteria



POWER: Central Sensitization — Amplifies normal gut stretch signals into severe pain via the amygdala and ACC.
LORE: Low-dose TCAs are used for neuromodulation of pain pathways, not at doses for clinical depression.

Nutrition Nadia | Nutritional Support

STAT: MUST Score ≥ 2



POWER: Intracellular Shift — Sudden insulin release from feeding forces phosphate into cells, causing cardiac failure.
LORE: Never restrict protein in cirrhosis; late evening snacks actually reduce overnight muscle catabolism.

Oesophageal Cancer Oscar | SCC vs Adeno

STAT: 8:1 Male (Adeno)



POWER: Field Cancerization — Head/neck SCC predicts oesophageal SCC due to shared mucosal exposures.
LORE: Obesity is strongly linked to adenocarcinoma through mechanical reflux pressure and hormonal adipokine overdrive.

GI NETs Greta | Neuroendocrine Tumours

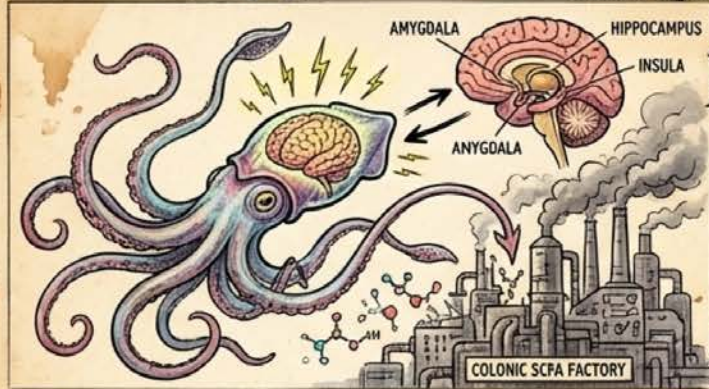
STAT: Ki67 $< 3\%$ (G1)



POWER: AMP Secretion — VIPomas activate adenylyl cyclase causing massive ($> 3\text{L/day}$) watery diarrhoea.
LORE: PRRT (TfR-DOTATATE) uniquely targets the somatostatin receptors universally expressed on midgut NETs.

Gut-Brain Axis Gregory | Bidirectional Comms

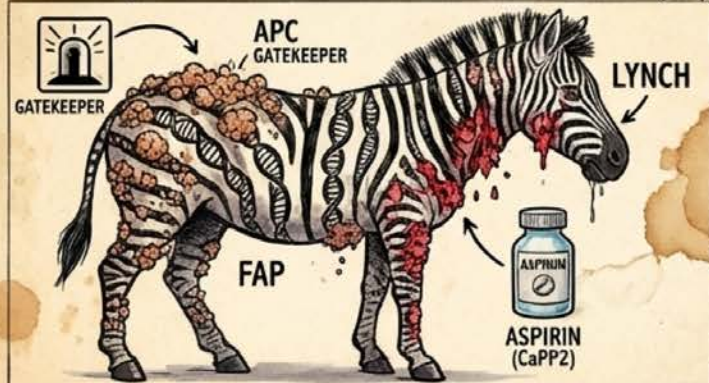
STAT: 80%
Afferent Vagus



POWER: Vagal Anti-Inflammation — Alpha7-nicotinic ACh receptors on macrophages shut down systemic cytokines.
LORE: Psychological stress directly triggers mast cell activation and altered motility, proving bidirectional IBS links.

Hereditary Cancers Heidi | FAP & Lynch

STAT: 100%
CRC Risk (FAP)



POWER: APC Gatekeeper — Germline loss guarantees hundreds of adenomas by teenage years.
LORE: Aspirin chemoprotection (CaPP2) drastically reduces colorectal cancer risk in Lynch syndrome.



Bariatric Norma | Metabolic Surgery

STAT: 60-80%
T2DM Remission



POWER: Incretin Surge — Bypassing the proximal bowel alters bile acids and hormones, fixing metabolism.
LORE: T2DM resolves in Rous-en-Y before significant weight loss, proving the effect is hormonal, not just caloric.

EoE EOS | Eosinophilic Oesophagitis

STAT: ≥15 Eosinophils/HPF



POWER: Th2 Remodelling — IL-5 and IL-13 destroy the epithelial barrier, causing strictures and rings.
LORE: A normal oesophagus has absolutely zero eosinophils; their presence always indicates pathology.

Portal HTN Phoebe | Varices & TIPSS

STAT: Target HVPG <12



POWER: Portosystemic Shunt — TIPSS directly decompresses the system but floods the brain with ammonia (HE risk).
LORE: Non-selective beta-blockers (carvedilol) reduce portal pressure by causing splanchnic vasoconstriction.

Liver Transplant Lance | End-Stage Cure

STAT: King's College pH <7.25



POWER: Super-Urgent Listing — Reverses fulminant paracetamol ALF before fatal cerebral oedema occurs.
LORE: A standalone arterial pH <7.25 is enough to mandate super-urgent transplant listing in paracetamol overdose.

The GI Attending | Master Review Boss

STAT: 3 Integrated Cases



POWER: Integrative Synthesis — Connects molecular mechanism, trial evidence, and anatomical reasoning into one plan.
LORE: True mastery is knowing why PSC-IBD causes CCA, why SONIC dictates dual therapy, and when to TIPSS.

